

Diclogesic Suppositories®

12.5mg For children. (Diclofenac sodium)

Description:

White to off-white torpedo-shaped rectal suppositories

Pharmacodynamics/ Pharmacokinetics:

Diclofenac sodium has shown antipyretic, analgesic and anti-inflammatory activity. It is believed that inhibition of prostaglandin synthesis may be involved in the anti-inflammatory effect.

Diclogesic suppositories 12.5mg is readily absorbed from the rectum within less than one hour. The metabolites are mainly excreted in the urine and small amounts are excreted in the bile.

Indication:

Rheumatoid arthritis, osteoarthritis and ankylosing spondylitis; non-articular rheumatism, acute attack of gout, acute musculo-skeletal disorders, low back pain, post-traumatic conditions and pain, pain and inflammation in orthopaedic, dental and other minor surgery. Children: Juvenile chronic arthritis.

Dosage and administration:

In children from (1 – 5) years: 1mg/kg body weight 3 times daily

In children from (5 – 10) years: 3mg/kg body weight 3 times daily

For rectal use only.

Dosage

As general recommendation, the dose should be individually adjusted. Adverse effects may be minimized by using the lowest effective dose for the shortest duration necessary to control symptoms (see section WARNINGS AND PRECAUTIONS)

Established cardiovascular disease or significant cardiovascular risk factors

Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤100 mg daily if treated for more than 4 weeks (see section WARNINGS AND PRECAUTIONS).

Contraindications:

Diclofenac sodium is contraindicated in patients with known hypersensitive to the drug or any of its ingredients, or in patients who develop asthmatic attacks or acute rhinitis and urticaria. It is also contraindicated in children less than one year old and in children hypersensitive to salicylic acid or prostaglandin synthesis inhibiting agents.

Diclofenac sodium is contraindicated in severe cardiac failure (see section WARNINGS AND PRECAUTIONS).

Warnings and Precautions:

- Gastrointestinal bleeding or ulceration can occur at any time during treatment with or without warning symptoms or a previous history, they generally have more serious consequences in the elderly, however, in these rare cases where gastrointestinal bleeding or ulceration occur in patients receiving diclofenac the drug should be discontinued

- As with other NSAIDs allergic reactions, including anaphylactic/ anaphylactoid reactions, including anaphylactic/ anaphylactoid reactions could also occur in rare cases without earlier exposure to the drug

- As with other NSAIDs values of one or more liver enzymes may be increased during prolonged treatment with diclofenac, monitoring of hepatic

- Function is indicated as precautionary measure. If abnormal liver function tests, persist or worsen diclofenac should be withdrawn

- Caution is indicated in the elderly on basic medical grounds, in particular, it is recommended that the lowest effective dosage should be used in frail elderly patients or those with low body weight

- Pregnancy diclofenac should be employed only for compelling reasons and only in the lowest effective doses

- This applies particularly to the last three months of pregnancy (owing to the possibility of uterine inertia and/ or premature closure of ductus arteriosus).

- Nursing mothers: the active substances pass into the breast milk, but in quantities so small that no undesirable effects on the infants are to be expected

- Severe cutaneous reactions, including Stevens - Johnson syndrome and toxic epidermal necrolysis (Lyell's syndrome), have been reported with diclofenac sodium. Patients treated with diclofenac sodium should be closely monitored for signs of hypersensitivity reactions. Discontinue diclofenac sodium immediately if rash occurs.

- Adverse effects: Dermatological: Occasional - rashes or skin eruptions Cases of hair loss, bullous eruptions, erythema multiforme, Stevens- Johnson syndrome, toxic epidermal necrolysis (Lyell's syndrome), and photosensitivity reactions have been reported.

Cardiovascular Effects: Treatment with NSAIDs including diclofenac, particularly at high dose and in long term, maybe associated with an increased risk of serious cardiovascular thrombotic events (including myocardial infarction and stroke).

Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (eg. Hypertension, hyperlipidaemia, diabetes mellitus and

smoking) should be treated with diclofenac only after careful consideration and only at doses ≤ 100 mg daily when treatment continue for more than 4 weeks.

As the cardiovascular risks of diclofenac may increase with dose and duration of exposure, the lowest effective daily dose should be used for the shortest duration possible. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially when treatment continues for more than 4 weeks. Patients should remain alert for the signs and symptoms of serious arteriothrombotic events (e.g. chest pain, shortness of breath, weakness, slurring of speech), which can occur without warning. Patients should be instructed to see a physician immediately in case of such an event.

Interactions with other medications:

- Aspirin: Concomitant administration of diclofenac and aspirin is not recommended because diclofenac is displaced from its binding sites during the concomitant administration with aspirin resulting in lower plasma concentrations, peak plasma levels and AUC values.

- Digoxin, methotrexate and cyclosporin: Diclofenac like other NSAIDs, may increase the toxicity of these drugs. Patients should be observed closely particularly if renal function is impaired.

- Lithium: Diclofenac decreases lithium renal clearance and increases its plasma levels. In patients taking diclofenac and lithium concomitantly, lithium toxicity may develop.

- Oral Hypoglycemics: Diclofenac does not alter glucose metabolism in normal subjects nor does it alter the effects of oral hypoglycemic agents. However, there are rare reports of changes in effects of insulin or oral hypoglycemic agents in the presence of diclofenac that necessitated changes in doses of such agents.

- Diuretics: Diclofenac and other NSAIDs may inhibit the activity of diuretics. Concomitant treatment with potassium sparing diuretics may be associated with an increase in serum potassium levels.

Statement on usage during pregnancy and lactation:

Not applicable

Adverse effects/ Side effects:

Diclogesic suppositories 12.5mg is well tolerated; the most common side effects are headache, drowsiness, dizziness and rash.

Cardiac disorders

Uncommon*: Myocardial infarction, cardiac failure, palpitations, chest pain.

*The frequency reflects data from long-term treatment with a high dose (150mg/day)

Description of selected adverse drug reactions

Arteriothrombotic events

Meta-analysis and pharmacoepidemiological data point towards an increased risk of arteriothrombotic events (for example myocardial infarction) associated with the use of diclofenac, particularly at a high dose (150mg daily) and during long-term treatment (see section WARNINGS AND PRECAUTIONS).

Overdosage and Treatment:

In the event of acute overdosage, it is recommended that the stomach be emptied by vomiting or gastric lavage. Forced diuresis may be beneficial because the drug is excreted in the urine.

The effect of dialysis or hemoperfusion in the elimination of diclofenac remains unproven. In addition to supportive measures, the use of oral activated charcoal may help to reduce the absorption of diclofenac.

Storage conditions:

Do not store above 30°C

Dosage forms and packaging available:

Each suppository contains 12.5mg diclofenac sodium

10 suppositories per box;

Shelf-life: 3 years

Manufactured by:-

Dar Al Dawa Development & Investment Co., Ltd.

P.O. Box 9364, Amman, 11191, Jordan.

Product Registration Holder:

Antah Pharma Sdn. Bhd.

No. 3, Jalan 13/1,

46300 Petaling Jaya,

Selangor, Malaysia.

Date of Revision of Package insert: October 2021

Keep medicament out of reach of children. Jauhikan daripada kanak-kanak.

THIS IS A MEDICAMENT

- Medicament is a product which affects your health, and its consumption contrary to instructions is dangerous for you.
- Follow strictly the doctor's prescription, method of use and the instructions of the pharmacist who sold the medicament
- The doctor and the pharmacist are experts in medicine, its benefits and risks
- Do not by yourself interrupt the period of treatment prescribed for you
- Do not repeat the same prescription without consulting your doctor

10/2021

دار الدواء
Dar AlDawa

Diclogesic Suppositories® Insert Malaysia

Color
 Pantone Black U
Size: 205X135 mm
Item Code 4170513034
Pharma Code: 727